



Tests you can trust

Name : Sankeerthi K (20Y/F)

Date : 01 Mar 2025

Test Asked : Executive Health Checkup Below 40 Yrs With Utsh

Report Status: Interim Report



First National Diagnostic Chain to have
100% of its Labs with NABL Accreditation[#]



Temperature-
Controlled
Sample Logistics



Unique Barcode
Tracking



Fully Automated
Machines Inspected
Daily



Abnormal Values
Re-Checked Twice



Reports Verified By Expert
MD Pathologists Stationed
at Every Lab



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Scan the QR code inside the report to check authenticity
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QR code will remain active for 30 days from report release date

Accredited by



NABL From 2005



ISO 9001: 2015 – From 2015



CAP From 2007

PROCESSED AT :**Thyrocare**

H. NO. 1-9-645,Vidyanagar,
Adikmet Road,Near SBH,
Hyderabad-500 044



Tests you can trust

📍 Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 📞 9870666333 ✉ wellness@thyrocare.com

First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

NAME : SANKEERTHI K (20Y/F)
REF. BY : HCL
TEST ASKED : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH
UTSH

SAMPLE COLLECTED AT :
, H. NO. 5-9-244/247, PLOT NO. 247SHAILI
GARDENS, YAPRAL, JAWAHAR NAGAR,
HYDERABAD, TELANGANA, INDIA,

Report Availability Summary

Note: Please refer to the table below for status of your tests.

✅ **8** Ready 🟡 **0** Ready with Cancellation 🔄 **2** Processing ❌ **0** Cancelled in Lab

TEST DETAILS

REPORT STATUS

EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

Partially Ready ✅

FASTING BLOOD SUGAR(GLUCOSE)

Ready ✅

HEMOGRAM

Ready ✅

URIC ACID

Ready ✅

TSH - ULTRASENSITIVE

Ready ✅

COMPLETE URINE ANALYSIS

Processing 🔄

HBA PROFILE

Ready ✅

LIPID PROFILE

Processing 🔄

KIDPRO WITHOUT CALCIUM

Ready ✅

SERUM SGOT-SGPT RATIO

Ready ✅

BILIRUBIN - TOTAL

Ready ✅

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NAME : SANKEERTHI K (20Y/F)**REF. BY** : HCL**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH**SAMPLE COLLECTED AT :**

, H. NO. 5-9-244/247, PLOT NO. 247SHAILI GARDENS,
YAPRAL, JAWAHAR NAGAR, HYDERABAD, TELANGANA,
INDIA,

MOBILE NO :**PAN ID** :**DOB** :**Summary Report****Tests outside reference range**

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
EOSINOPHILS	7.3	%	1-6
MEAN CORP.HEMO.CONC(MCHC)	28.9	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	23.9	pq	27.0-32.0
MEAN CORPUSCULAR VOLUME(MCV)	82.7	fL	83.0-101.0
RED CELL DISTRIBUTION WIDTH (RDW-CV)	17.9	%	11.6-14.0
TOTAL RBC	5.27	X 10 ⁶ /μL	3.8-4.8
DIABETES			
AVERAGE BLOOD GLUCOSE (ABG)	132	mg/dL	90-120
HbA1c	6.2	%	< 5.7
LIVER			
ALANINE TRANSAMINASE (SGPT)	44.96	U/L	< 34
RENAL			
BLOOD UREA NITROGEN (BUN)	7.63	mg/dL	7.94 - 20.07
UREA (CALCULATED)	16.33	mg/dL	Adult : 17-43

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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WITH UTSH

SAMPLE COLLECTED AT :
, H. NO. 5-9-244/247, PLOT NO. 247SHAILI
GARDENS, YAPRAL, JAWAHAR NAGAR, HYDERABAD,
TELANGANA, INDIA,

PATIENTID : SK25888300 **MOBILE NO** : **DOB** : **PAN ID** :

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	91.03	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :

The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed, icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT) : 01 Mar 2025 08:00
Sample Received on (SRT) : 02 Mar 2025 01:10
Report Released on (RRT) : 02 Mar 2025 05:01
Sample Type : FLUORIDE PLASMA
Labcode : 0103122554/HCL18
Barcode : DI625567



Amulya

Dr Amulya MD (Path)

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NAME : SANKEERTHI K (20Y/F)**REF. BY** : HCL**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH
UTSH**PATIENTID** : SK25888300**MOBILE NO** :**DOB** :**SAMPLE COLLECTED AT :**

, H. NO. 5-9-244/247, PLOT NO. 247SHAILI
GARDENS, YAPRAL, JAWAHAR NAGAR,
HYDERABAD, TELANGANA, INDIA,

PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)	H.P.L.C	6.2	%

Bio. Ref. Interval. :**Bio. Ref. Interval.: As per ADA Guidelines**

Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic

Guidance For Known Diabetics

Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	132	mg/dL
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Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HbA1c values**Please correlate with clinical conditions.****Sample Collected on (SCT)** :01 Mar 2025 08:00**Sample Received on (SRT)** : 02 Mar 2025 01:11**Report Released on (RRT)** : 02 Mar 2025 05:21**Sample Type** : EDTA Whole Blood**Labcode** : 0103122667/HCL18**Barcode** : DB584030

Dr Amulya MD (Path)



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NAME : SANKEERTHI K (20Y/F)
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TEST ASKED : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH

SAMPLE COLLECTED AT :
, H. NO. 5-9-244/247, PLOT NO. 247SHAILI
GARDENS, YAPRAL, JAWAHAR NAGAR,
HYDERABAD, TELANGANA, INDIA,

PATIENTID : SK25888300 **MOBILE NO** : **DOB** : **PAN ID** :

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	12.6	g/dL	12.0-15.0
Hematocrit (PCV)	CPH Detection	43.6	%	36.0-46.0
Total RBC	HF & EI	5.27	X 10⁶/μL	3.8-4.8
Mean Corpuscular Volume (MCV)	Calculated	82.7	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	23.9	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	28.9	g/dL	31.5-34.5
Red Cell Distribution Width (RDW - CV)	Calculated	17.9	%	11.6-14.0
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	6.03	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	48.6	%	40-80
Lymphocytes Percentage	Flow Cytometry	37.1	%	20-40
Monocytes Percentage	Flow Cytometry	6.3	%	2-10
Eosinophils Percentage	Flow Cytometry	7.3	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.1	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.93	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.24	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.38	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.03	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.44	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.01	X 10 ³ / μL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.006	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	170	X 10 ³ / μL	150-410

Clinical history is asked for all the relevant abnormalities detected and in absence / failure of receiving of clinical history, results are rechecked twice and released. Advised clinical correlation.

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

Sample Collected on (SCT) : 01 Mar 2025 08:00
Sample Received on (SRT) : 02 Mar 2025 01:11
Report Released on (RRT) : 02 Mar 2025 05:21
Sample Type : EDTA Whole Blood
Labcode : 0103122667/HCL18
Barcode : DB584030

Dr Amulya MD (Path)

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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]**NAME** : SANKEERTHI K (20Y/F)**REF. BY** : HCL**TEST ASKED** : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH
UTSH**PATIENTID** : SK25888300**MOBILE NO** :**DOB** :**SAMPLE COLLECTED AT :**, H. NO. 5-9-244/247, PLOT NO. 247SHAILI GARDENS,
YAPRAL, JAWAHAR NAGAR, HYDERABAD, TELANGANA,
INDIA,**PAN ID** :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	7.63	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.58	mg/dL	0.55-1.02
BUN / SR.CREATININE RATIO	CALCULATED	13.15	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	16.33	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	28.15	Ratio	< 52
URIC ACID	PHOTOMETRY	4.38	mg/dL	3.2 - 6.1

Please correlate with clinical conditions.**Method :**

BUN - Kinetic UV Assay.

SCRE - Creatinine Enzymatic Method

B/CR - Derived from serum Bun and Creatinine values

UREAC - Derived from BUN Value.

UR/CR - Derived from UREA and Sr.Creatinine values.

URIC - Uricase / Peroxidase Method

Sample Collected on (SCT) : 01 Mar 2025 08:00**Sample Received on (SRT)** : 02 Mar 2025 01:11**Report Released on (RRT)** : 02 Mar 2025 04:19**Sample Type** : SERUM**Labcode** : 0103122663/HCL18**Barcode** : DF263363

Dr Amulya MD (Path)

PROCESSED AT :**Thyrocare**

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UTSH**PATIENTID** : SK25888300**MOBILE NO** :**DOB** :**SAMPLE COLLECTED AT :**, H. NO. 5-9-244/247, PLOT NO. 247SHAILI GARDENS,
YAPRAL, JAWAHAR NAGAR, HYDERABAD, TELANGANA,
INDIA,**PAN ID** :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BILIRUBIN - TOTAL	PHOTOMETRY	0.41	mg/dL	0.3-1.2
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	28.48	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	44.96	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	0.63	Ratio	< 2

Please correlate with clinical conditions.**Method :**

BILT - Vanadate Oxidation

SGOT - IFCC* Without Pyridoxal Phosphate Activation

SGPT - IFCC* Without Pyridoxal Phosphate Activation

OT/PT - Derived from SGOT and SGPT values.

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NAME : SANKEERTHI K (20Y/F) **SAMPLE COLLECTED AT :**
REF. BY : HCL , H. NO. 5-9-244/247, PLOT NO. 247SHAILI GARDENS,
TEST ASKED : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS WITH UTSH YAPRAL, JAWAHAR NAGAR, HYDERABAD, TELANGANA,
INDIA,
PATIENTID : SK25888300 **PAN ID** :
MOBILE NO :
DOB :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TSH - ULTRASENSITIVE	E.C.L.I.A	1.55	μIU/mL	0.54-5.30

Comments :

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Pregnancy reference ranges for TSH/USTSH :

Trimester || T3 (ng/dl) || T4 (μg/dl) || TSH/USTSH (μIU/ml)

1st || 83.9-196.6 || 4.4-11.5 || 0.1-2.5

2nd || 86.1-217.4 || 4.9-12.2 || 0.2-3.0

3rd || 79.9-186 || 5.1-13.2 || 0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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Sample Received on (SRT) : 02 Mar 2025 01:11
Report Released on (RRT) : 02 Mar 2025 04:19
Sample Type : SERUM
Labcode : 0103122663/HCL18 Dr Amulya MD (Path)
Barcode : DF263363

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NAME : SANKEERTHI K (20Y/F) **SAMPLE COLLECTED AT :**
REF. BY : HCL , H. NO. 5-9-244/247, PLOT NO. 247SHAILI
TEST ASKED : EXECUTIVE HEALTH CHECKUP BELOW 40 YRS GARDENS, YAPRAL, JAWAHAR NAGAR, HYDERABAD,
WITH UTSH TELANGANA, INDIA,

PATIENTID : SK25888300 **MOBILE NO** : **DOB** : **PAN ID** :

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	133	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

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Sample Type : SERUM
Labcode : 0103122663/HCL18 Dr Amulya MD (Path)
Barcode : DF263363



Amulya

CONDITIONS OF REPORTING

- ❖ The reported results are for information and interpretation of the referring doctor only.
- ❖ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ❖ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ❖ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ❖ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ❖ This report is not valid for medico-legal purpose.
- ❖ Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.

EXPLANATIONS

- ❖ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ❖ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ❖ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ❖ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ❖ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ❖ **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- ❖ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ❖ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ❖ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ❖ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

+T&C Apply, #As on 5th December 2024, *As per a survey on doctors' perception of laboratory diagnostics (IJARIIT,2023)